



TRANSTHORACIC ECHOCARDIOGRAPHY REPORT

Name:	TOPPAZZINI, KYLE	Height:	183 cm	Date	2025-10-16 11:59:30 AM
Patient ID:	4919634891	Weight:	96 kg	Sonographer:	SA
Birthdate:	Sep 02, 1970	BSA:	2.18	Ref. Phys	Mohammed Nassoh (West) Hafez
Age	55y	BP:	118/72	Location:	Kanata
Sex	Male				

Conclusion/Interpretation:

1. Technically difficult study. IV Definity contrast was used to enhance endocardial visualization.
2. Sinus rhythm (100 bpm).
3. The left ventricle is moderately dilated with normal wall thickness. Severe systolic dysfunction. LVEF = 29%. Global hypokinesis.
4. Right ventricular size and systolic function.
5. No significant valvular abnormalities.
6. The right ventricular systolic pressure cannot be estimated due to the trivial amount of tricuspid regurgitation.
7. Minor changes compared to the study from 2025.

Dr. Preeti Anand

Confirmed by : Preeti Anand, panand, Oct 27, 2025

Findings:

Indication: Left ventricular dysfunction.

ECG Rhythm: Sinus rhythm (100 bpm).

Study Quality: This was a technically difficult study with suboptimal views. Ultrasound enhancing agent study was performed for left ventricular opacification.

Left Ventricle: The left ventricle is moderately dilated. Left ventricular wall thickness is normal. Severe systolic dysfunction. The calculated ejection fraction by Simpson's method is 28.8%. Global hypokinesis. Indeterminate diastolic function.

Right Ventricle: The right ventricle is normal in size and function.

Left Atrium: Left atrium is normal size by volume.

Right Atrium: Right atrium is normal size.

ASD/VSD: No clear evidence of interatrial communication by color flow doppler analysis.

Aortic Valve: The aortic valve is trileaflet and appears structurally normal. No aortic stenosis. No aortic regurgitation.

Mitral Valve: Normal appearing mitral valve with no evidence of mitral valve stenosis. Trace mitral valve regurgitation.

Tricuspid Valve: The tricuspid valve appears structurally normal. Trace tricuspid valve regurgitation. Unable to estimate RVSP due to inadequate TR jet spectral doppler profile.

Pulmonic Valve: Pulmonic valve appears structurally normal. Trace pulmonic valve regurgitation. No significant pulmonic stenosis.

Aorta: The aortic root (3.6 cm) and ascending aorta (3.3 cm) are normal in size when indexed to BSA.

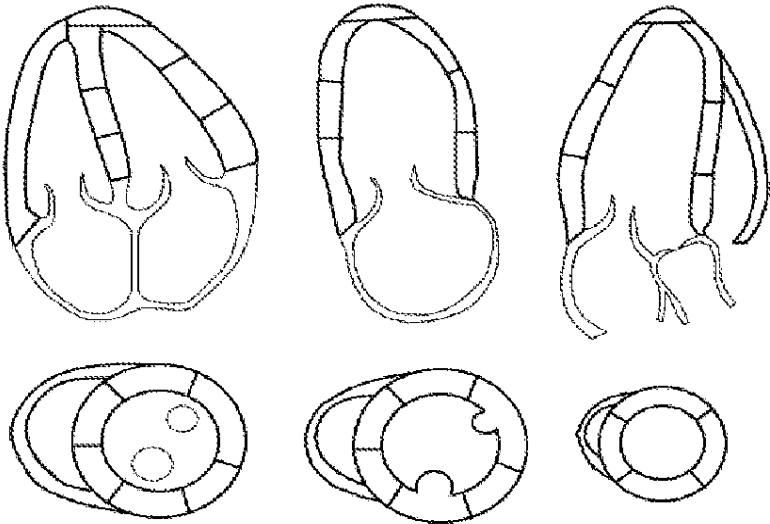
Pulmonary Artery:

IVC/Hepatic Veins: The inferior vena cava is normal with normal inspiratory collapse.

Pericardium: No pericardial effusion noted.

Send To List: 27022 Abhinav Iyengar;28813 Mohammed Nassoh (West) Hafez;

Rest



Not Visualized 2 - Hypokinetic 5 - Aneurysmal
1 - Normal 3 - Akinetic 4 - Dyskinetic

Cardiac Dimensions/Volumes:				Doppler Measurements:		
EDV-4ch	210	ml	56 - 155	MV E Vel	0.57	m/s
EDV-2ch	217	ml	56 - 155	MV A Vel	0.85	m/s
ESV-4ch	156	ml	19 - 58	MV E/A Ratio	0.7	
EF Biplane	28.8	%		MV Decel Time	88	ms
IVSd	0.9	cm	(< 1.1)	E` Lateral	0.05	m/s
LVIDd	6.4	cm	(< 5.8)	E/E` Avg	13.8	
LVPWd	1	cm	(< 1.1)	E/E`	17.2	
LV Mass	258	g		RAP	3	mmHg
LVd Mass Ind (ASE)	118	g/m2	(< 115.00)			
Relative Wall Thickness	0.31		(< 0.420)			
LVIDs	5.4	cm	(< 4.1)			
Ao Diam	3.6	cm	(< 3.7)			
Ao asc	3.3	cm	(< 3.5)			
LA Diam	3.9	cm	M:3.0 - 4.0 F:2.7 - 3.8			
% FS	15.6	%	30 - 45			
LAVI	20	ml/m2	(< 35)			
RA Area	14.4	cm2				
TAPSE	1.55	cm				

(dimensions and normal values are dependent on gender and BSA. See ASE guidelines for specific reference values)